



OFFICIAL

NMHS MHS PROCEDURE

Medication: Reconciliation and Review

Scope (Audience)	All NMHS Mental Health Services (MHS) Clinical Staff
Scope (Area)	All NMHS MHS Sites
Summary	This procedure outlines the minimum requirements and outlines clear roles and responsibilities for a patient's medication review on admission, during the admission, transfer to another health service or discharge.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Medication Safety (Std 4) Communicating for Safety (Std 6)
Procedure Sponsors	Director of Medical Services and Chief Pharmacist MHSDHS
Procedure Contact	Chief Pharmacist MHSDHS
Version	4.0
Review Date	18/03/2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This procedure outlines the minimum requirements for a patient's medication review on admission, during the admission, transfer to another health service or discharge. This procedure outlines clear roles and responsibilities in relation to medication review and provides guidance on how to perform a medication review and complements the [WA Medication Review Policy MP0104/19](#) and the [Best Practice Principles for Medication Review](#).

2. Background

Medication reconciliation and review is a multidisciplinary responsibility and must be patient centred to ensure safe and effective use of medications at all stages of the medication management pathway. This activity ensures continuity of care, reduce the risk of preventable medication-related adverse effects and improve patient safety.

3. Risk

Communication issues between health professions and care facilities can cause medication errors and preventable adverse drug events. Unintentional changes to patients' medication regimens often happen during hospital admissions and at discharge.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Procedure Principles

4.1 Standard 1: Medication Reconciliation on Admission

Medication Reconciliation on Admission Medication reconciliation, is required for all inpatients, completed by a credentialed professional by the end of next calendar day after admission and balanced against patient risk. Patients admitted to HiTH services requires a medication reconciliation completed within 48 hours of after admission.

4.1.1 Performing a medication reconciliation

1. Obtains a BPMH (including prescribed or non-prescribed medications, non-prescription medications and complementary supplements) and ADR/allergy history.
2. The BPMH must include the generic medication name, dosage, frequency, route and any recently altered or ceased medications, and if the patient is using a dose administration aid or on interval supply.
3. Sources of medication history can include but not limited to:
 - a referral letter
 - patient interview
 - patient's own medication (POMs)

- patient's current medication list
- patient's My Health Record
- carer/next of kin/case worker
- recent discharge summary
- community mental health service
- general practitioner
- community pharmacy
- private psychiatrist or other clinicians

4. Confirms the medication history with at least one other source:

- One source of medication information is sufficient if the patient is transferred from a facility where medication administration is controlled and a current medication list is available e.g. hostel, prison, nursing home, hospice, long-term inpatient in a hospital, or known non-adherence.
- This does not include transfer from emergency departments, or short-term hospital stays unless medication reconciliation has been documented, and evidence provided.
- If a second source is deemed unnecessary, this must be explicitly documented.

4.1.2 Role of Admitting Medical Officers

- The medical officer will perform a medication reconciliation as per the procedure outlined in Section 5.1.1 on admission
- The medication history and source of information are documented in the patient record
- The appropriate medication is charted for the patient.
- ADR or allergies are documented in the medication chart.

4.1.3 Role of Clinical Pharmacists

- The clinical pharmacist will perform a medication reconciliation independent from the medical officer as per the procedure outlined in Section 5.1.1 within end of next calendar day of admission.
- Review the medication history and compare the history obtained to the medical officer's admission medication list.
- The information is documented on the Medication History and Management Plan (MMP) either electronically or manually (PMR 60F).
- Any discrepancies are raised with the treating team and documented in the patient record and MMP.
- The MMP is kept with the medication charts during the patient's admission and filed with the charts on discharge or transfer. For HiTH patients the MMP is filed in the integrated progress notes.
- Completes the 'Medicines taken prior to presentation to hospital' on the front of the WA Hospital Medication Chart (WA HMC PMR 60A) – 'see WA MMP' if used.
- Adds any omitted ADR Alerts to the relevant section of the chart.

4.1.4 Role of Nursing Staff

- If the admission takes place after hours and the patient appears to be taking high risk medications, nursing staff must notify the clinical pharmacist to allow for priority medication reconciliation.

- Notification should be via email to Graylands.Pharmacists@health.wa.gov.au with the subject heading “New Admission on High Risk Medication – dd/mm/yyyy”

4.2 Standard 2: Medication Chart Review

All current medication charts for inpatients or Best Practice® profiles for HiTH patients should be reviewed regularly by the patient’s medical team, nurse administering the medication and clinical pharmacist throughout a patient’s admission to ensure the provision of optimal treatment to the patient. This is necessary in order to review the effectiveness, utility and potential harm of each medication on its own and the regimen as a whole.

This includes medications to treat current medical conditions and de-prescribing medication that are no longer required.

4.2.1 Prescriber chart review

Where feasible, a daily medication chart review should be undertaken by the patient’s prescriber. See Section 5.5 if the prescriber is unable to complete medication chart review for all patients. For medication reviews:

- Generic medication names must be used except when it is appropriate to use trade names. See [Appendix 4 in Medication: Prescribing Administration and Management for Inpatient](#) for guidance.
- Indication must be documented for prescribed, regular and pro re nata (PRN) medication.
- Prescription must be legible and legal.
- Appropriate doses for medication prescribed.
- Dosing times are appropriate.
- Dosing forms are clarified.
- No error-prone abbreviations must be used. [Appendix 1 in Medication: Prescribing Administration and Management for Inpatient](#) for guidance.
- Medication is prescribed according to the service guidelines. [Formulary One](#); [NMHS MH Medication: Formulary and Non-Formulary Prescribing Policy](#); [NMHS MH Medication: Off-Label Prescribing including Prescribing Greater than maximum Dose Policy](#).
- Review chart for medication interactions.

A medication review must also be completed before commencing a new medication or deprescribing a medication. The prescriber needs to ensure the alteration of the medication regimen will not cause interaction with the current medications which may interfere with the patient’s management and adhere to the service guidelines.

4.2.2 Clinical pharmacist chart review

All patients admitted to hospital for inpatient care should have a review of their medication chart or Best Practice profile completed by a clinical pharmacist by the end of each day, where feasible. See Section 5.5 if **clinical** pharmacist is unable to complete medication chart review for all patients. The chart review will include:

- Identifying, clarifying, monitoring and assessing medications prescribed for potential adverse drug reactions.
- Ensuring medication orders meet legal requirement

- Identifying changes in dose, frequency, formulation and route of administration to regular medications and assessing the appropriateness for the patient.
- Providing clarification of:
 - medication names from trade names to generic for exceptions refer to NMHS MH guidelines on prescribing medication with generic and/or trade names
 - doses, dosing times for all medications
 - Any error prone abbreviations used
- Monitoring the patient's response to medications such as therapeutic drug monitoring and biochemistry parameters
- Identifying new medications - providing or arranging for education.
- Documenting the review in the appropriate signoff box on the medication chart. (e.g. The Pharmaceutical Review daily sign-off box on the WA HMC, pharmacy sign off on Clozapine Titration chart.)

4.2.3 Reconciliation of newly written medication charts

The prescribing doctor:

Reconciles medication chart ensuring accurate documentation of current prescribed medications and ensuring any changes are clearly documented on the ceased medication order.

The clinical pharmacist:

Reconciles all newly written medication charts against the patient's previous medication chart and any discrepancies are brought to the attention of the prescribing doctor.

4.2.4 Chart review at the time of administration

The chart needs to be reviewed by a clinician before administering medication to the patient within their scope of clinical practice.

Prior to administering a medication to the patient:

- The 6 rights of Safe Medication Administration need to occur: Review whether there are any existing interactions
- Check the medication order is legible and clear
- Confirm the indication to administer a PRN medication

The medication chart needs to be signed once the medication is administered.

4.3 Standard 3: Provision of Medication Education to Patients

Patients and/or carers must be provided with appropriate education and information about their medication.

NMHS MHS uses [Choice and Medication](#)® printable leaflets for written medication information on psychotropic medicines, available in various forms. [AusDI](#)® has Consumer Medication Information (CMIs) in more detail. [Formulary One](#) has simplified Medication Information Leaflets (MILs) for a range of non-psychotropics medications.

Medication education and information can be provided as either written and/or verbal to the patient and/or carer. This allows the patient/carer to make informed choices regarding their medication treatment.

Medication education provided must be documented in the patient's progress notes and in the discharge summary.

All patients require a medication profile or list on discharge or on transfer to another site.

4.4 Standard 4: Medication Reconciliation on Transfer or Discharge/Communication with Community Clinicians

Medication handover is required on patient transfer or discharge with receiving healthcare provider as per the [DoH Clinical Handover Policy \(MP0095/18\)](#). The discharge summary is the primary document communicating a patient's care plan to healthcare professionals taking over the care of the patient following hospital transfer or discharge from the service.

4.4.1 Medication reconciliation at discharge

The patient's clinical pharmacist is notified as soon as possible about the patient's discharge or transfer date to assist in the discharge or transfer planning for the patient. The clinical pharmacist should be involved in the medication component of the discharge summary wherever possible.

The clinical pharmacist:

1. Discusses discrepancies with the prescriber and makes changes in consultation with the prescriber as required.
2. Enters the medications and ADRs/allergies into the eMedications and eAllergies, including the explanations for changes in therapy.
3. Enters details of the community pharmacy if a dose administration aid (DAA) and/or reduced interval dispensing is organised on discharge
4. Checks and signs that the prescriptions on discharge or transfer match the current medication profile.
5. Ensures the patient record (NaCS/WA MMP) contains evidence that the medication reconciliation process has occurred.
6. Ensures any relevant POMs are transferred, returned or safely disposed of on discharge or transfer as outlined in the NMHS MH - [Medication: Patient's Own Medication Policy and Procedure](#).
7. Details, if required (e.g. when the next Long-Acting Antipsychotic depot is due; next blood test for patients on clozapine, medication plan for clinicians post discharge) should be documented or any other relevant information.
8. Provides prescriptions and/or medication list profile to relevant community pharmacy if the prescription is not dispensed by the hospital pharmacy.
9. Liaise with the relevant community pharmacy if preparation of DAA and/or reduced interval dispensing is required.
10. Provide additional verbal communication to the receiving healthcare provider if required.

The medical officer:

1. Prepares a discharge summary using the NaCS including reasons for medication treatment changes, ongoing monitoring and management requirements and medication alerts that is sent to the patient's follow-up clinicians at the time of discharge.
2. Confirms the reconciled medication list and prescription/s

3. Generates reconciled prescriptions using the eMedications.
4. Includes a duplicate copy of the clinician discharge summary in the patient record.
5. Provide additional verbal communication to the receiving healthcare provider if required.

4.4.2 Hospital or service transfer

On transfer to a different hospital or service the nurse needs to ensure following should be transferred:

- Copy of all medication charts
- Copy of completed WA MMP
- Reconciled Medication List
- Discharge summary.

NOTE: Patients transferred from another NMHS MH inpatient service or from a HiTH service under the same admission do not need another medication reconciliation performed. A copy of the current MMP, medication charts or eMedication list needs to be included in the transfer process.

The final responsibility for authorising the discharge summary resides with the Team Consultant Psychiatrist.

4.5 Patient Risk Assessment and Prioritisation

Clinical staff are to use clinical judgement to determine prioritisation for medication review (medication reconciliation on admission and discharge/transfer, medication chart review and patient education) if daily medication reviews for all patients are not achievable. Risk assessment of patient factors is required to identify patients at greater risk of medication related harm and priority for medication review is provided to this cohort of patients.

5. Monitoring and Evaluation

Compliance with this procedure is mandatory.

The NMHS MHS Audit and Medication Safety Pharmacist or nominated staff member will contribute to the statewide biannual WA Medication Reconciliation Audit, as per the Medication Reconciliation Audit Guidelines using the mandated [WA Medication Reconciliation Electronic Audit Tool](#) ([downtime form](#)).

The report can be obtained via [WA Medication Reconciliation Audit Dashboard](#) (via Power BI®) for NMHS Safety Quality and Governance, NMHS MHS Drug and Therapeutics Committee and the MHS Management Committee.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

6. Definitions

Terms	Definitions
Medication Reconciliation	The formal process of obtaining a complete and accurate list of a patient's current home medications and comparing the medical officer's admission, transfer or discharge orders to that list.
Best Possible Medication History (BPMH)	A list of all the medicines a patient is taking prior to admission (prescribed and non-prescribed) and obtained from interviewing the patient and/or their carer (where possible) and confirmed using a number of different sources of information.
Medication History and Management Plan (WA MMP)	A standardised medicine reconciliation form used for hospital admission and discharge.
HiTH	Hospital in the Home
Notifications and Clinical Summaries (NaCS)	A web-based application for managing patient discharge summary information and prescriptions in the Common Information Model provided by Department of Health Integration Hub services.
Credential Professional	A health professional (namely medical doctor, nurse, pharmacist, pharmacy staff) who has the relevant knowledge and training in taking medication histories, medication review and/or providing medication education to patient

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms

7. Related Documents

WA Health mandatory policies	• WA Health Best Practice Principles for Medication Review • WA Health Medication Reconciliation • WA Health Medication History and Management Plan Guidelines • WA Health Medication Review Policy MP 0104/19 • WA Health Medication Reconciliation Audit Tool Guidelines • WA Health Clinical Handover Policy MP0095/18
NMHS Policy documents	• NMHS Policy – Discharge Summary
NMHS MHS Policy documents	• Medication: Off-Label Prescribing Including Prescribing Greater than Maximum Dose • Medication: Formulary and Non-Formulary Prescribing
Standards	• NSQHS Standards Second Edition 2021
Other	• WA Notification and Clinical Summaries (NaCS®) • Choice and Medication

8. Document Control

Version	Policy author	Summary of changes	Approval date
V1.0	NMHS MH Chief Pharmacist	MHEG – Meeting minutes	14/04/2010
V1.2	NMHS MH Chief Pharmacist	Scheduled Review: New template, medication reconciliation is the responsibility of clinical pharmacists, removed "or other credentialed staff".	07/08/2018
V1.3	NMHS MH Chief Pharmacist	Links updated; New WA Medication Review Policy linked.	27/08/2020
V2.0	NMHS MH Chief Pharmacist	New template; title change to include 'review' in line with the WA Medication Review Policy – updated information regarding review.	11/08/2022
V3.0	NMHS MH Chief Pharmacist	New Template Review for policy to be in line with WA Medication Review Policy, WA Health Best Practice Principles for Medication Review, WA Health Clinical Handover Policy, WA Health Medication Reconciliation Audit Tool Guidelines	01/07/2025
V4.0	NMHS MH Chief Pharmacist	Scheduled review, new template, updated links; change to procedure.	18/03/2026

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community. ISD number: 5145.

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